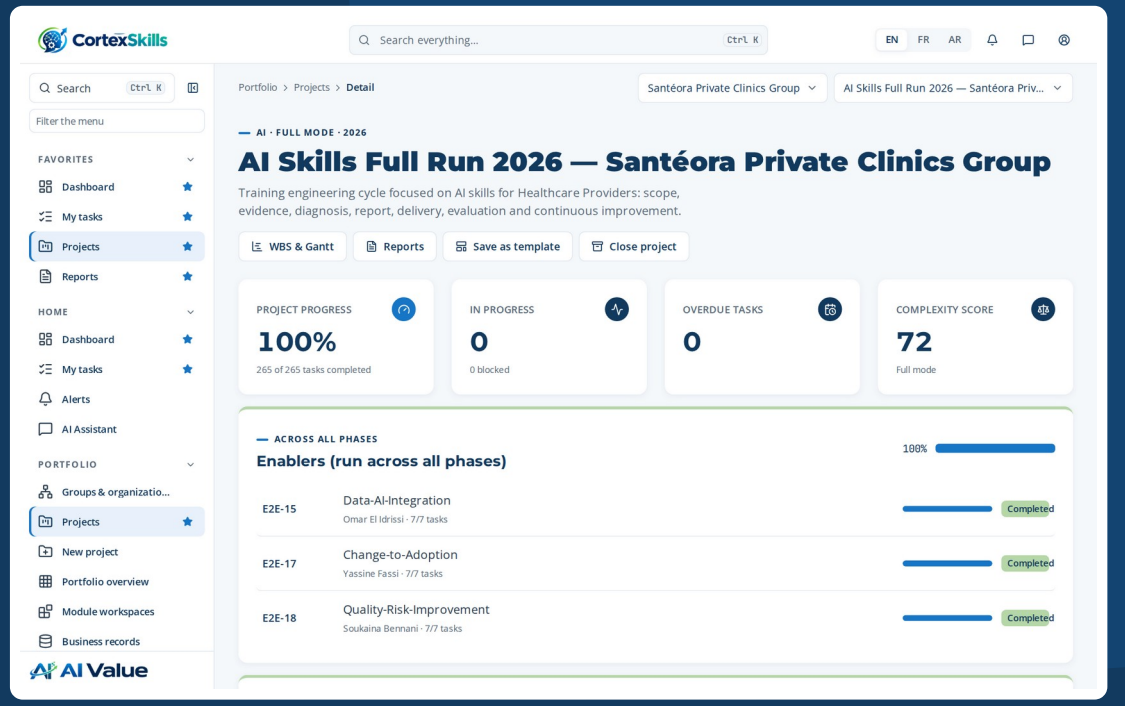
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FULL CASE · HEALTHCARE · 2026

# AI skills training engineering at a private clinic group

Santéora Private Clinics Group: the project, the Training Engineering Report, the Training Plan and every document produced with CortexSkills




MEASURABLE · GOVERNED · SUSTAINABLE

AGENDA

# How the case is presented

01

## The clinic group and the project

Context, charter, scope, method, schedule

02

## Diagnosis

Questionnaires, maturity, gaps, needs

03

## Training Engineering Report

Structure, findings, roadmap, risks

04

## Training Plan

Programs, agendas, design, budget

05

## Delivery, evaluation and audit

Sessions, Kirkpatrick, ROI, audit

06

## Documents produced

37 published documents, traced

EXECUTIVE SUMMARY

# A complete AI skills cycle, documented end to end

Santéora Private Clinics Group · 2,600 employees · AI Skills Full Run 2026 — Santéora Private Clinics Group

**100 %**
of the run completed — 35 end-to-end processes

**35 / 40**
people answered the questionnaires (88 %)

**6**
trainings in 2 programs, prioritized by impact

**1,260,000**
training budget; 1,238,497 spent

## What the case shows

- Diagnosis: “Information systems” is the weakest axis (2 / 5); the largest skill gap is “AI bed and staff capacity planning” (average gap 3.0).
- 10 training needs were validated and grouped into 6 themes, deployed in 4 roadmap initiatives over three waves.
- Kirkpatrick results 77 % · 74 % · 75 %; training ROI 108 % (target ≥ 100%).
- 37 documents published, 1,077 pages; the Training Engineering Report alone has 193 pages.

01

# The clinic group and the project

Context, charter, scope, method and schedule

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CONTEXT

# Santéora Private Clinics Group

Santéora Private Clinics Group operates in the Healthcare Providers sector, in Casablanca, with 2600 employees.

| Topic | Content |
|---|---|
| **Vision** | Become the reference of Healthcare Providers in the region, with teams trained to the best standards. |
| **Mission** | Deliver safe, compliant service through skilled people and controlled processes. |
| **Values** | Commitment · Excellence · Sharing |
| **Context** | Strong competition, stricter ISO 7101 audits and the arrival of AI triage and clinical decision support. |
| **Products — existing** | Core services of Clinical Care & Nursing |
| **Products — new** | Digital services supported by AI triage and clinical decision support |
| **Market segments** | Large accounts · Public sector · Individual customers |

## Organization and headcount

| Function | People |
|---|---|
| Operations | 3 |
| Quality & HSE | 3 |
| Finance | 2 |
| Human Resources | 2 |
| Sales & Marketing | 2 |
| Information Systems | 2 |
| Clinical Care & Nursing | 2 |

# Key dates and SWOT of the clinic group

## 2008

Company founded

## 2014

Second site opened

## 2019

ISO 7101 certification

## 2025

Digital transformation program launched

### Strengths

- Experienced, committed team
- Good knowledge of the market
- Stable processes

### Areas for improvement

- Low use of AI triage and clinical decision support
- Reporting kept in spreadsheets
- Low use of AI medical imaging assistance
- Little versatility between roles
- Low use of AI bed and staff capacity planning

### Opportunities

- Demand for digital services
- Training contracts refunded by OFPPT

### Threats

- Stricter ISO 7101 audits
- Departure of key experts

*Source: TER sections 1.3 and 1.4.*

# AI Skills Full Run 2026 — Santéora Private Clinics Group

Training engineering cycle focused on AI skills for Healthcare Providers: scope, evidence, diagnosis, report, delivery, evaluation and continuous improvement.

**72**
complexity score (out of 100): full run with gates

**40**
stakeholders registered and consulted

**17**
scope criteria approved in the Scope of Work

**MS · MO · OP**
decision levels covered (strategic, middle, operational)

## Strategic objectives

- SO-1 — Adopt AI triage and clinical decision support across Clinical Care & Nursing by end 2027 (horizon 2027-12-31)
- SO-2 — Zero major findings in ISO 7101 audits (horizon 2026-12-31)

# Functions and decision levels in scope

Focus: AI · Decision levels: MS, MO, OP · Approval: Approved (2026-04-30, Mehdi Lahlou)

| Function | MS — strategic | MO — middle | OP — operational |
|---|---|---|---|
| **Operations** | ● | ● | ● |
| **Quality & HSE** | ● | ● | — |
| **Finance** | ● | — | ● |
| **Human Resources** | — | ● | ● |
| **Sales & Marketing** | ● | ● | ● |
| **Information Systems** | ● | ● | — |
| **Clinical Care & Nursing** | ● | — | ● |

## How the scope is set

Each function × decision-level cell is a weighted criterion of the Scope of Work. 16 criteria were approved by the sponsor before any questionnaire was sent; every later table of the report is filtered by this scope.

*Source: TER section 1.8.*

METHOD

# Six phases, each closed by a gate with its deliverables

The training engineering mission follows the phases below, each closed by a gate with its deliverables.

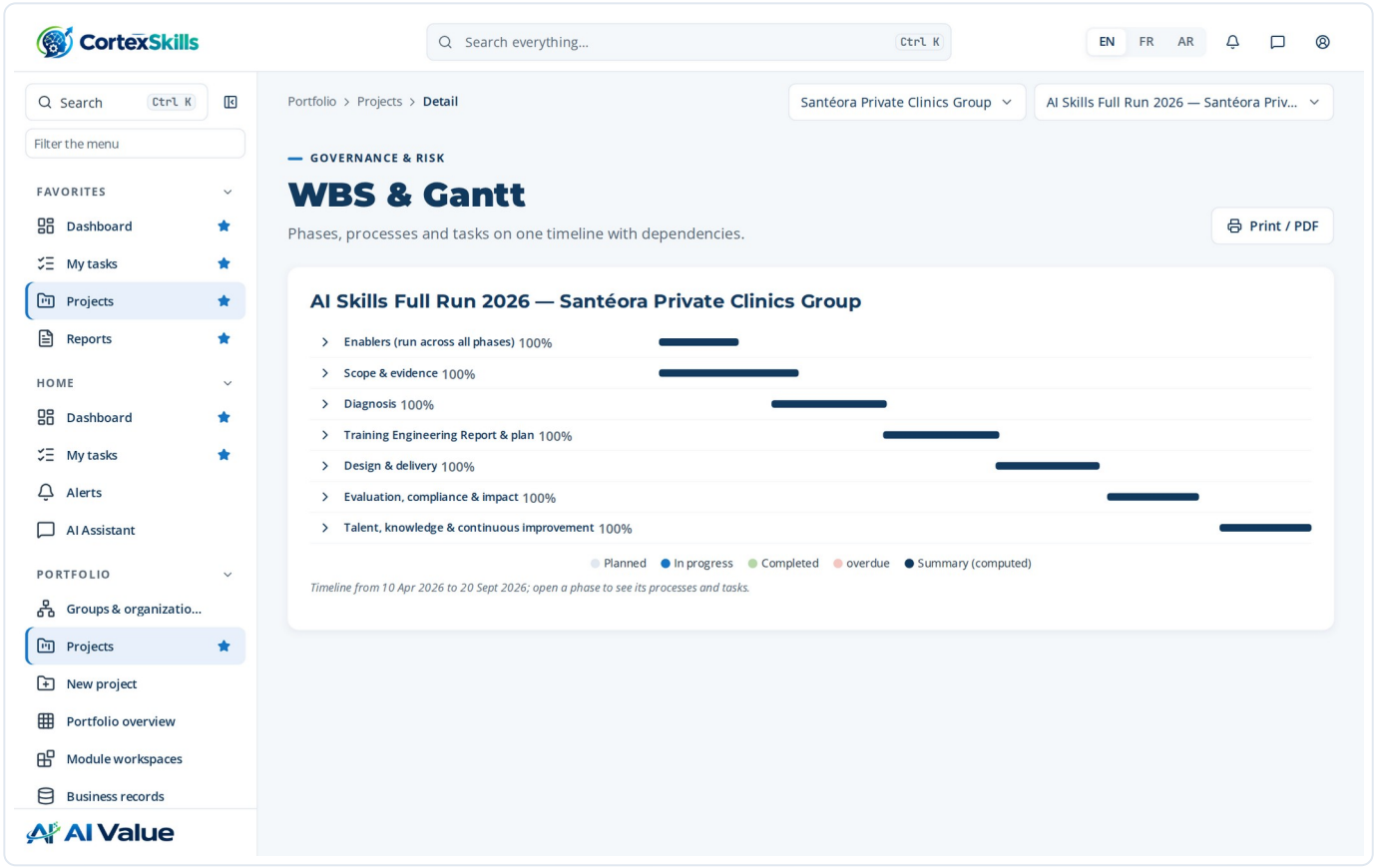
| Phase | End-to-end processes | Progress |
|---|---|---|
| **1. Scope & evidence** | E2E-32 · E2E-33 · E2E-19 · E2E-24 | 100% |
| **2. Diagnosis** | E2E-20 · E2E-25 · E2E-26 · E2E-27 · E2E-28 · E2E-29 · E2E-02 · E2E-21 | 100% |
| **3. Training Engineering Report & plan** | E2E-01 · E2E-03 · E2E-30 · E2E-22 · E2E-23 · E2E-16 · HCPR-E2E-02 | 100% |
| **4. Design & delivery** | E2E-04 · E2E-05 · E2E-06 · E2E-13 · E2E-14 | 100% |
| **5. Evaluation, compliance & impact** | E2E-07 · E2E-08 · E2E-09 · HCPR-E2E-01 | 100% |
| **6. Talent, knowledge & continuous improvement** | E2E-10 · E2E-11 · E2E-12 · E2E-31 | 100% |

*Source: Training Engineering Report, section 1.5 "The current mission".*

# Work packages and the Gantt chart

| Work package | Start | End | Progress |
|---|---|---|---|
| Scope & evidence | 2026-04-10 | 2026-05-07 | 100 % |
| Diagnosis | 2026-05-08 | 2026-06-04 | 100 % |
| Report & plan | 2026-06-05 | 2026-07-02 | 100 % |

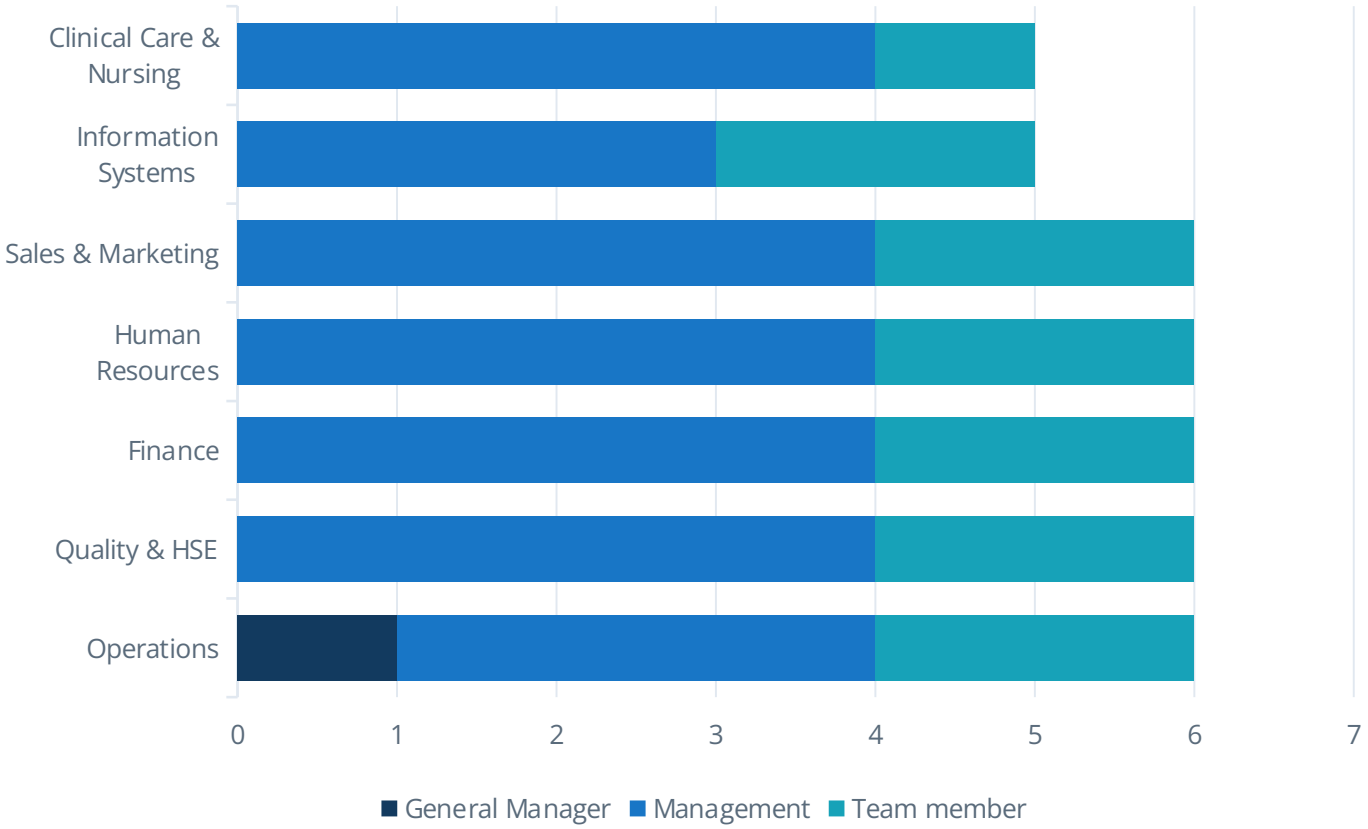
| Sessions | Date |
|---|---|
| S-01 AI triage and clinical decision … | 2026-07-19 |
| S-02 AI medical imaging assistance | 2026-07-26 |
| S-03 AI triage and clinical decision … | 2026-08-02 |
| S-04 AI medical imaging assistance | 2026-08-09 |




*WBS and Gantt of the project in CortexSkills.*

# 40 people consulted across every function

40 people were consulted: 1 from general management, 26 managers and 13 staff members.




*People consulted by function and population (TER 1.7).*

| Channel | People |
|---|---|
| Face-to-face | 1 |
| Combination | 26 |
| Email | 13 |

Consent recorded for 40 of 40 people before any answer was stored.

# 02

# Diagnosis

Who answered, where the clinic group stands, which skills are missing

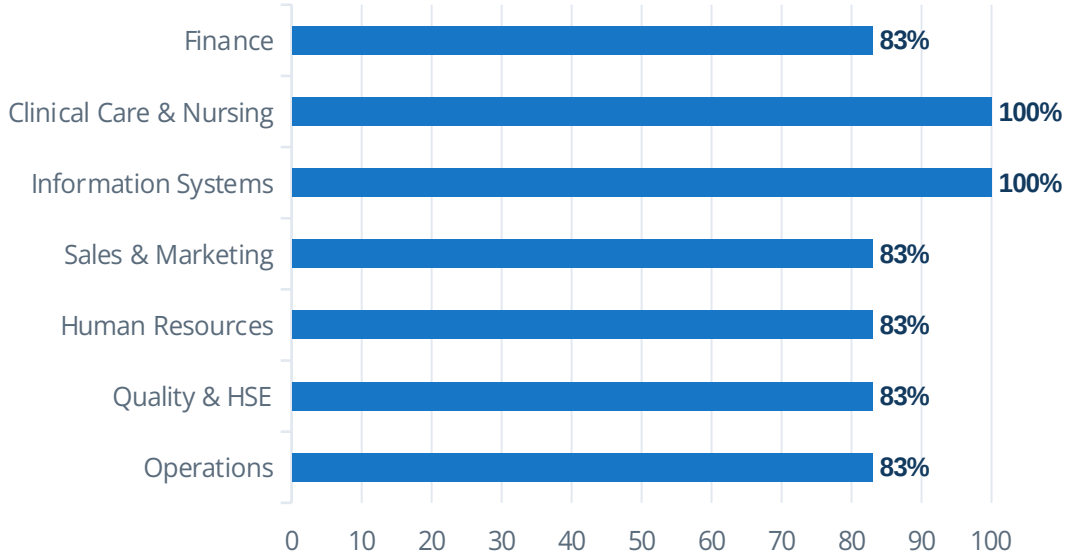
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QUESTIONNAIRE CAMPAIGN

# 35 of 40 people answered

40 people were invited to answer; 35 answered.

| Function | General Manager | Management | Team member | Responded |
|---|---|---|---|---|
| Operations | 1 | 3 | 2 | 5 |
| Quality & HSE | 0 | 4 | 2 | 5 |
| Human Resources | 0 | 4 | 2 | 5 |
| Sales & Marketing | 0 | 4 | 2 | 5 |
| Information Systems | 0 | 3 | 2 | 5 |
| Clinical Care & Nursing | 0 | 4 | 1 | 5 |
| Finance | 0 | 4 | 2 | 5 |




*Response rate by function.*

| Channel | Invited | Responded | Rate | Completeness |
|---|---|---|---|---|
| Application | 18 | 18 | 100 % | 95 % |
| Face-to-face | 13 | 13 | 100 % | 89 % |
| Email | 4 | 4 | 100 % | 89 % |

*Source: TER sections 1.6, 1.12 and 1.13.*

# What the teams said about the last training plan

### What did you notice about the n-1 training plan?

• Good participation, but little follow-up after the sessions.
• Trainings were chosen late in the year and with little link to our objectives.

### What were the strengths of the n-1 plan?

• Sessions held on site, close to the teams.
• Practical trainers who knew our sector.

### What improvements did the n-1 plan bring?

• Supervisors use the new procedures.
• Fewer errors on ISO 7101 records.

### What were the challenges of the n-1 plan?

• Measuring the effect of the trainings.
• Releasing people from the job during peaks.

### What did you appreciate and like about the n-1 plan?

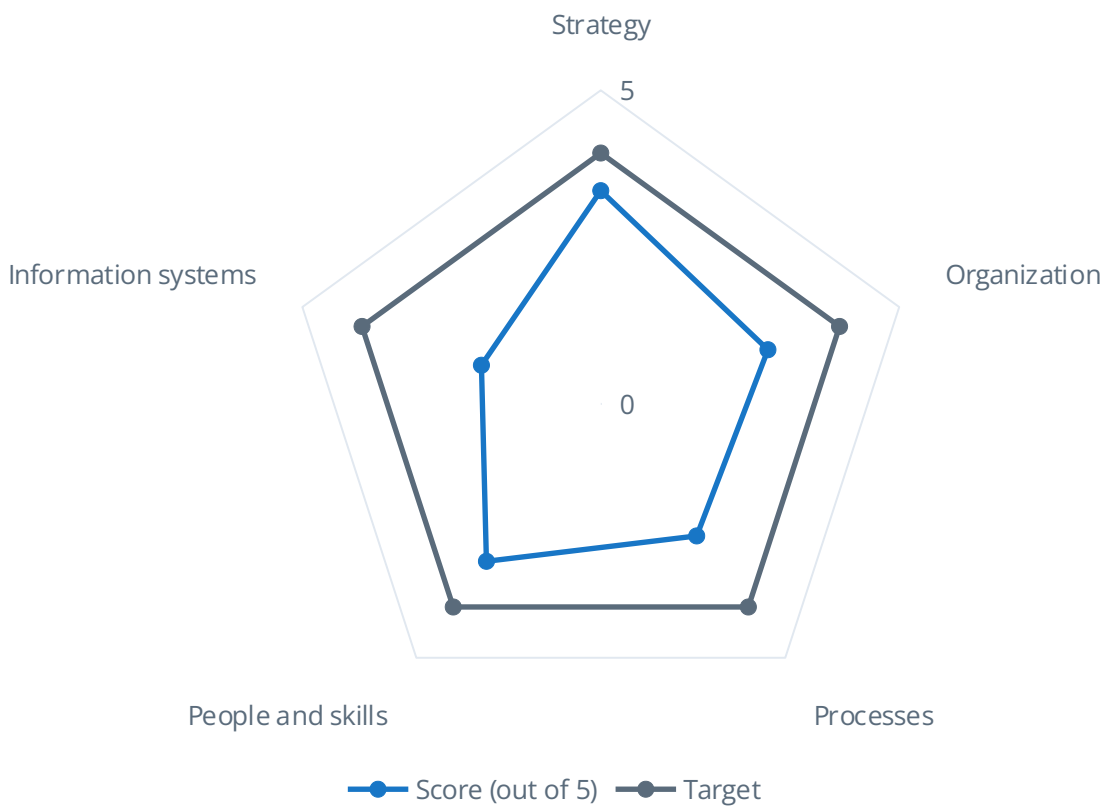
• The workshops on real cases.
• The exchange between sites.

### For a new training plan, what would you like to see done differently?

• Shorter half-day formats with a workshop each time.
• A plan built from the needs of each function.

*Source: TER chapter 2 — 62 trainings attended are listed in section 2.1.*

# Five axes scored, maturity one level below target




*Diagnostic score by axis against target level 4 (TER 1.9).*

| Axis | Score | Reading |
|---|---|---|
| Strategy | 3.4 | Strength to build on |
| Organization | 2.8 | Partly in place |
| Processes | 2.6 | Partly in place |
| People and skills | 3.1 | Partly in place |
| Information systems | 2 | Priority for development |

| Model | Current | Target | Gap |
|---|---|---|---|
| CMM maturity (CMM) | 3 | 4 | 1 |
| Leadership maturity (Leadership) | 3 | 4 | 1 |

*Maturity models (TER 1.10).*

# Where each function stands at each decision level

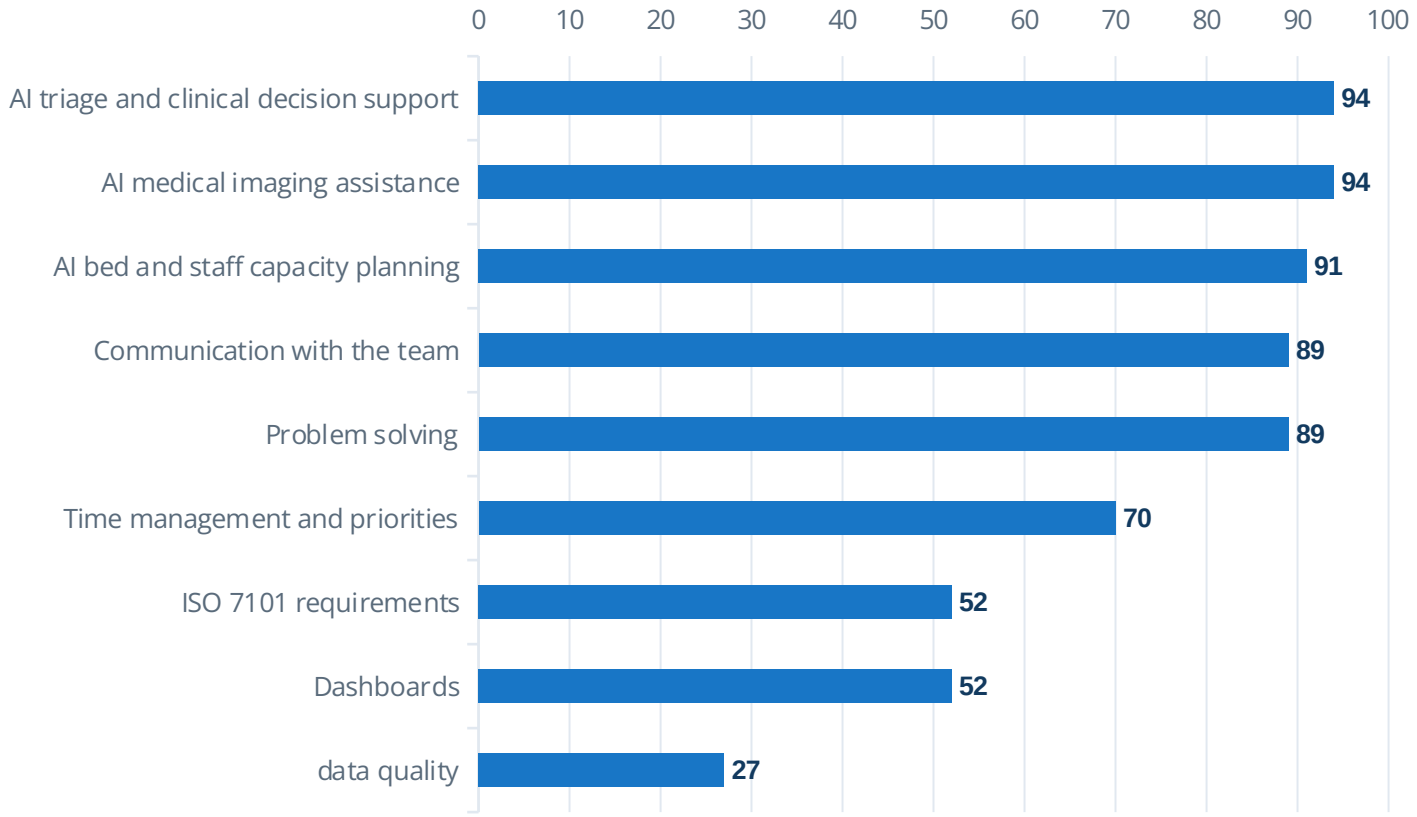
Performance by decision level: MS strategic management, MO operational management, OP operational.

| Function | MS | MO | OP | Main gaps |
|---|---|---|---|---|
| **Operations** | 88% Amber | 83% Amber | 76% Amber | AI triage and clinical decision support · AI medical imaging assistance · AI bed and staff cap… |
| **Quality & HSE** | 76% Amber | 52% Red | 86% Amber | AI medical imaging assistance · AI bed and staff capacity planning · AI triage and clinical de… |
| **Finance** | 81% Amber | 58% Red | 77% Amber | AI bed and staff capacity planning · AI triage and clinical decision support · AI medical imag… |
| **Human Resources** | 79% Amber | 54% Red | 63% Red | AI triage and clinical decision support · AI medical imaging assistance · AI bed and staff cap… |
| **Sales & Marketing** | 88% Amber | 82% Amber | 59% Red | AI medical imaging assistance · AI bed and staff capacity planning · AI triage and clinical de… |
| **Information Systems** | 78% Amber | 64% Red | 71% Amber | AI bed and staff capacity planning · AI triage and clinical decision support · AI medical imag… |
| **Clinical Care & Nursing** | 88% Amber | 73% Amber | 53% Red | AI triage and clinical decision support · AI medical imaging assistance · AI bed and staff cap… |

*Maturity heatmap on the fixed red → green status scale (TER 3.2).*

# The competences people asked for, by number of mentions

Consolidated from 30 responses.



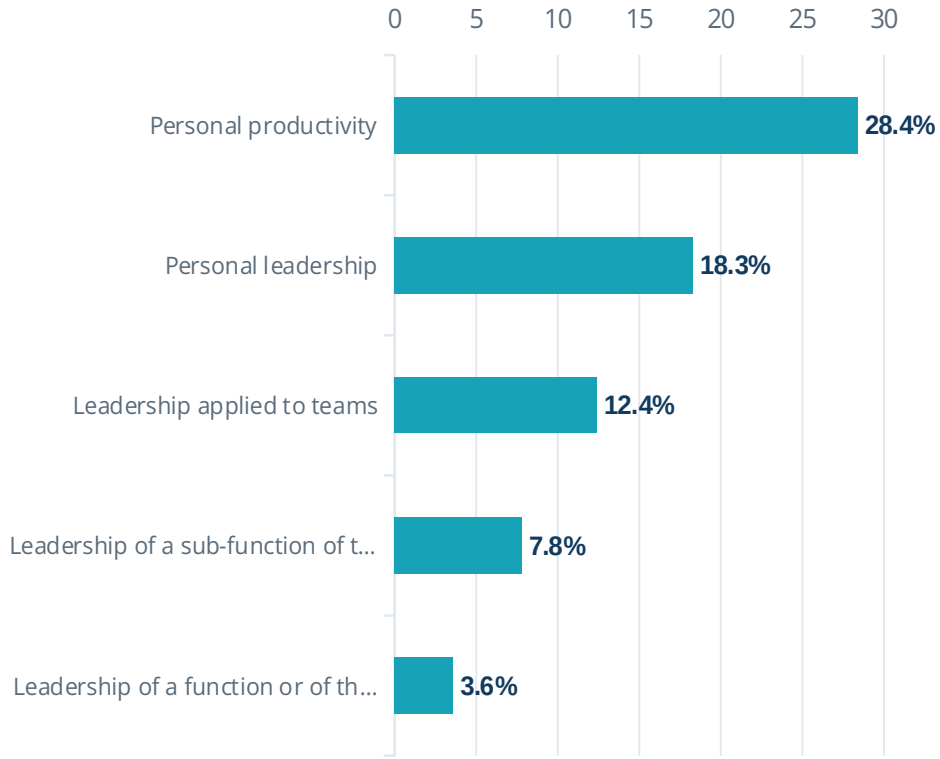

## Reading

The two AI clinical themes lead (94 and 94 mentions), followed by team communication and problem solving: the plan pairs each AI training with practical workshops on the clinics' own cases.

*Mentions across all answers (TER 3.3).*

# Self-assessment of the respondents, weakest first

| Competence | Average (1–5) | n |
|---|---|---|
| Rewards and motivation | 3.2 | 30 |
| Training and effectiveness | 3.2 | 30 |
| Positive thinking and positive results (focus on solutions, break a b… | 3.3 | 30 |
| Management by objectives and strategic vision applied to oneself | 3.3 | 30 |
| Communication with oneself | 3.3 | 29 |
| Communication with the team | 3.3 | 29 |
| Managing with flexibility | 3.3 | 28 |
| Management by objectives and strategic vision applied to others | 3.4 | 29 |

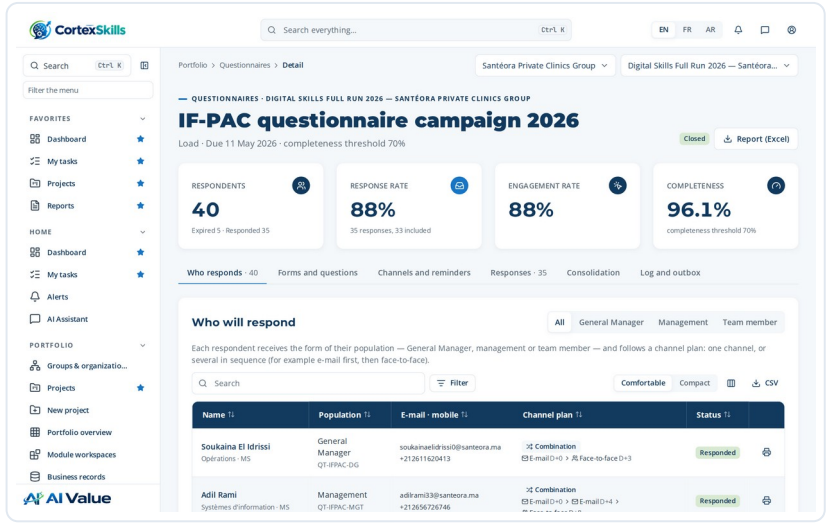



*Leadership levels reached (TER 3.7).*

# From measured gaps to a validated needs register

| Competency | People | Avg gap | Critical |
|---|---|---|---|
| AI medical imaging assistance | 3 | 2.0 | 0 |
| AI bed and staff capacity planning | 2 | 3.0 | 2 |
| AI triage and clinical decision support | 3 | 1.0 | 0 |

*Skill gaps by competency (TER 3.40).*




| # | Theme | Source | Impact | Status |
|---|---|---|---|---|
| 1 | AI bed and staff capacity planning | Diagnostic | 5 | Validated |
| 2 | Responsible AI and data protection | Skills gap | 5 | Validated |
| 3 | AI bed and staff capacity planning | GPEC | 5 | Validated |
| 4 | AI medical imaging assistance | Demand | 4 | Validated |
| 5 | Prompting and generative AI at work | SWOT | 4 | Validated |
| 6 | AI medical imaging assistance | Diagnostic | 4 | Validated |
| 7 | AI triage and clinical decision support | Skills gap | 3 | Validated |
| 8 | AI literacy for all staff | GPEC | 3 | Validated |
| 9 | AI triage and clinical decision support | Demand | 3 | Validated |
| 10 | AI literacy for all staff | SWOT | 3 | Validated |

*Training needs register (TER 3.41): every need traced to its source.*

# Clinical Care & Nursing

Needs, assessment and competences of the function that carries the strategic objective SO-1

| Decision level | Score (%) | Status | Gap |
|---|---|---|---|
| MS | 88 % | Amber | AI triage and clinical decision support |
| MO | 73 % | Amber | AI medical imaging assistance |
| OP | 53 % | Red | AI bed and staff capacity planning |

| Respondent | Population | Strengths / Areas for improvement | Competences |
|---|---|---|---|
| Anas Berrada | Management | Soins cliniques et infirmiers — Experienced, committed team — Low use… | AI bed and staff capacity planning |
| Anas Berrada | Management | Clinical Care & Nursing — Experienced, committed team — Little versat… | AI bed and staff capacity planning; Time management and priorities |
| Imane Bennani | Team member | Experienced, committed team — Reporting kept in spreadsheets | AI medical imaging assistance |
| Imane Bennani | Team member | Experienced, committed team — Little versatility between roles | AI medical imaging assistance; Time management and priorities |
| Hajar Mansouri | Management | Soins cliniques et infirmiers — Good knowledge of the market — Report… | AI bed and staff capacity planning |
| Hajar Mansouri | Management | Clinical Care & Nursing — Stable processes — Little versatility betwe… | AI bed and staff capacity planning; Time management and priorities |

*Source: TER sections 3.36 to 3.39.*

# How the teams use AI and what holds them back

### Current use

Daily, for triage and reports. (31)

### Barriers

Lack of training and unclear rules. (31)

### Risks to manage

Data protection and validation of results. (31)

### Priorities

AI medical imaging assistance; data quality (11)

AI triage and clinical decision support; data quality (10)

### Preferred format

Short workshops on the job, then coaching. (31)

| Strategic planning practice | Yes | No |
|---|---|---|
| Is there a strategic plan with macro business plans at company level? | 1 | 0 |
| Are this strategic plan and the macro business plans implemented? | 1 | 0 |
| Are they monitored? | 1 | 0 |
| Are they subject to continuous improvement? | 1 | 0 |

*Most frequent answers of 35 respondents (TER 3.45, 3.46).*

# What the respondents want the training to change

| Objective | SMART formulation | Mentions |
|---|---|---|
| **Improve customer satisfaction** | Satisfaction score ≥ 85% by end of year | 28 |
| **Reduce non-quality costs** | -20% rework in 12 months | 28 |
| **Develop human capital** | 80% of critical roles certified | 8 |

## Expected results

- Fewer errors and faster service.
- Teams able to use AI medical imaging assistance safely.
- Better audit results and staff retention.
- Teams able to use AI bed and staff capacity planning safely.
- Teams able to use AI triage and clinical decision support safely.

*Source: TER 3.42 (64 objectives stated) and 3.9.*

# 03

# Training Engineering Report

132 sections, 193 pages, generated from the run data

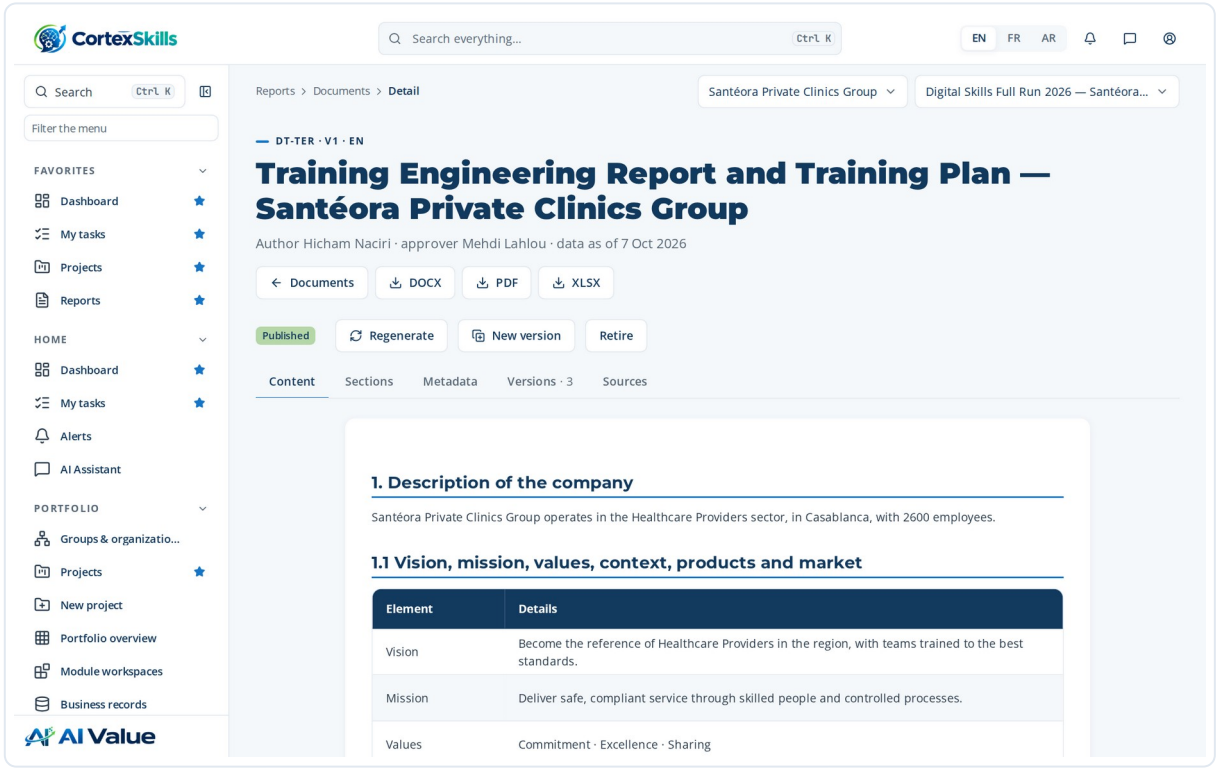
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# Training Engineering Report and Training Plan

Version 1 · author Hicham Naciri · approved by Mehdi Lahlou

| Chapter | Sections |
|---|---|
| 1. Description of the company | 14 |
| 2. The previous training plan | 2 |
| 3. Competence needs | 50 |
| 4. Perspectives | 3 |
| 5. The prioritized training plan | 23 |
| 6. Risks, controls and indicators | 2 |
| 7. Annexes — interview forms | 36 |

*Plus identification, data sources and approval.*




*The report in the document editor.*

# Needs linked to the strategy and to the HR orientations

Priority axes named by the General Management: Leadership and performance · AI triage and clinical decision support · ISO 7101 compliance · Dashboards

| ID | Objective | Horizon |
|---|---|---|
| SO-1 | Adopt AI triage and clinical decision support across Clinical Care & Nursing by end 2027 | 2027-12-31 |
| SO-2 | Zero major findings in ISO 7101 audits | 2026-12-31 |

## HR orientations

• Update job descriptions, individual objectives and the yearly assessment; build career paths.

## Change management

1) Focus and prioritization · 2) Stay positive · 3) Persevere and follow up with short daily repetitions · Patience · Lead by example · Progressiveness.

*Source: TER sections 3.1, 3.5 and 3.6.*

# Four initiatives deployed in three waves

The perspectives take into account the strategic orientations, the SWOT and improvement axes, the HR orientations and the recommendations made in the face-to-face meetin...

## Wave 1

- AI triage and clinical decision support — impact 5
- AI literacy for all staff — impact 5

## Wave 2

- AI medical imaging assistance — impact 4

## Wave 3

- AI bed and staff capacity planning — impact 3

| # | Improvement axis | Implemented | Wave |
|---|---|---|---|
| 1 | AI triage and clinical decision support | 60% | 1 |
| 2 | AI medical imaging assistance | 65% | 2 |
| 3 | AI bed and staff capacity planning | 70% | 3 |
| 4 | AI literacy for all staff | 75% | 1 |

*Source: TER chapter 4 and section 4.1.*

# Each risk reduced by named controls

| ID | Risk or opportunity | Category | Inherent | Residual | Controls |
|---|---|---|---|---|---|
| RISK-014 | Low learner and user adoption | Adoption | 16 | 8 | CTRL-081 |
| RISK-017 | Critical knowledge lost with expert departure | Operational | 16 | 8 | CTRL-070; CTRL-082 |
| RISK-001 | Unauthorized access to tenant data | Security | 20 | 6 | CTRL-001; CTRL-002; CTRL-048; CTRL-011 |
| RISK-005 | Training plan misaligned with strategy | Strategic | 16 | 6 | CTRL-049; CTRL-057; CTRL-076 |
| RISK-006 | Inaccurate skills data leading to wrong needs | Data quality | 15 | 6 | CTRL-064; CTRL-045 |
| RISK-030 | Position descriptions diverge from reality | Organizational | 12 | 6 | CTRL-066 |
| RISK-007 | Unverified AI content in the Training Engineering Report | AI | 16 | 5 | CTRL-041; CTRL-056 |
| RISK-009 | Expired mandatory certification | Compliance | 20 | 5 | CTRL-053; CTRL-054 |
| RISK-031 | Site deployment deviates from group plan | Operational | 12 | 5 | CTRL-071 |

*Top 9 of 35 risks by residual score (TER chapter 6).*

# Indicators followed every month during the run

| ID | Indicator | Target | Latest value | Status |
|---|---|---|---|---|
| KPI-003 | Critical gap rate | ≤ 10% | 2026-10: 8.2 | Green |
| KPI-004 | Training Engineering Report cycle time | ≤ 30 days | 2026-10: 24.6 | Green |
| KPI-005 | Design approval lead time | ≤ 20 days | 2026-10: 16.4 | Green |
| KPI-007 | Content publication lead time | ≤ 15 days | 2026-10: 12.3 | Green |
| KPI-008 | Session fill rate | ≥ 85% | 2026-10: 91.8 | Green |
| KPI-009 | Plan realization rate | ≥ 90% | 2026-10: 97.2 | Green |
| KPI-010 | Learner completion rate | ≥ 80% | 2026-10: 86.4 | Green |
| KPI-012 | Average Level 1 satisfaction | ≥ 4.2 | 2026-10: 4.5 | Green |
| KPI-013 | Skill progression rate | ≥ 70% | 2026-10: 75.6 | Green |
| KPI-015 | Training record correction rate | ≤ 1% | 2026-10: 0.82 | Green |

*10 of the 61 indicators of TER section 6.1.*

# 04

# Training Plan

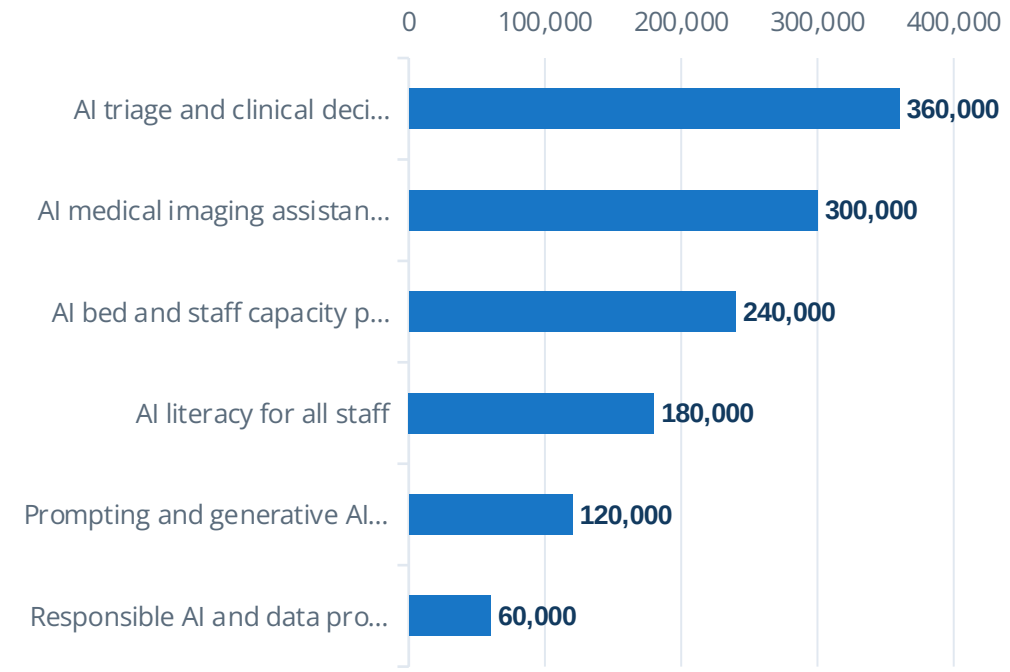
The plan 2026 holds 2 programs and 6 trainings, 12 days per group in total and 44 group-days to deliver, for a budget of 1,260,000.

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# Six themes ranked by impact and urgency

Plan 2026 — status Locked — budget 1,260,000.

| # | Theme | Days | Groups | Budget |
|---|---|---|---|---|
| 1 | AI triage and clinical decision support | 3 | 6 | 360,000 |
| 2 | AI medical imaging assistance | 2 | 4 | 300,000 |
| 3 | AI bed and staff capacity planning | 2.5 | 3 | 240,000 |
| 4 | AI literacy for all staff | 1.5 | 3 | 180,000 |
| 5 | Prompting and generative AI at work | 1 | 2 | 120,000 |
| 6 | Responsible AI and data protection | 2 | 2 | 60,000 |




*Budget by theme.*

# Two programs, six trainings, clear prerequisites

| Code | Program | Strategic axis | Trainings |
|---|---|---|---|
| PRG-1 | AI skills for Clinical Care & Nursing | Execution of the core function | TR-AI-01, TR-AI-02, TR-AI-03 |
| PRG-2 | AI foundations for all staff | Leadership, performance and responsible use | TR-AI-04, TR-AI-05, TR-AI-06 |

| ID | Training | Level | Duration | Prerequisites |
|---|---|---|---|---|
| TR-AI-01 | AI triage and clinical decision support | Intermediate | 3 day(s) | Basic digital literacy; the Foundation training is recommended |
| TR-AI-02 | AI medical imaging assistance | Intermediate | 2 day(s) | Basic digital literacy; the Foundation training is recommended |
| TR-AI-03 | AI bed and staff capacity planning | Advanced | 2.5 day(s) | TR-AI-01 or equivalent experience |
| TR-AI-04 | AI literacy for all staff | Foundation | 1.5 day(s) | None — open to all staff |
| TR-AI-05 | Prompting and generative AI at work | Foundation | 1 day(s) | None — open to all staff |
| TR-AI-06 | Responsible AI and data protection | Intermediate | 2 day(s) | Basic digital literacy; the Foundation training is recommended |

*Source: Training Plan "Programs" and TER section 5.1.*

TRAINING SHEET

# TR-AI-01 — AI triage and clinical decision support

Level Intermediate · 3 day(s) · program PRG-1

## Learning objectives

• Explain the key concepts of AI triage and clinical decision support and how they apply to Clinical Care & Nursing
• Apply the methods and tools of AI triage and clinical decision support to a real case from Santéora Private Clinics Group
• Plan the deployment of AI triage and clinical decision support with its indicators and controls (ISO 7101)

## Prerequisites

Basic digital literacy; the Foundation training is recommended

## Delivery

M1 AI triage and clinical decision support · E-learning · 2 h
M3 AI triage and clinical decision support · Virtual classroom · 3.5 h

DETAILED AGENDA

# AI triage and clinical decision support — half-day by half-day

22 activities over 3 day(s): lectures, quizzes and workshops on the clinics' own cases.

## Day 1 — morning

Lecture · Foundations of AI triage and clinical decision support: key c... (75 min)
Lecture · Why AI triage and clinical decision support matters for Clini... (45 min)
Quiz — key concepts of AI triage and clinical decision support (20 min)
Workshop — map today's practices in Clinical Care & Nursing a... (75 min)

## Day 1 — afternoon

Lecture · Methods and tools of AI triage and clinical decision support (75 min)
Lecture · Worked example from the Healthcare Providers sector (40 min)
Quiz — choosing the right method (20 min)
Workshop — apply the method to a real case from Santéora Priv... (80 min)

## Day 2 — morning

Lecture · Governance, risks and ISO 7101 requirements (60 min)
Lecture · Data, quality and evidence to keep (45 min)
Quiz — compliance and risk scenarios (20 min)
Workshop — build the control checklist of your process (80 min)

## Day 2 — afternoon

Lecture · Deploying AI triage and clinical decision support: roles, cha... (60 min)
Lecture · Indicators that measure the results (45 min)
Quiz — adoption and indicator cases (20 min)
Workshop — design the indicator dashboard of your team (80 min)

## Day 3 — morning

Lecture · Advanced practices and lessons learned in Clinical Care & Nur... (75 min)
Quiz — advanced cases (25 min)
Workshop — solve a critical incident scenario (100 min)

## Day 3 — afternoon

Lecture · Transfer to the workplace: the 30-60-90-day plan (60 min)
Quiz · Final assessment (Kirkpatrick level 2) (30 min)
Workshop — individual action plan and peer review (90 min)

# Why each audience should take the training

Training TR-AI-01 — behaviour, pain points and hopes of each persona

## Executive sponsor — General Manager

Decides on facts presented in short steering meetings; delegates execution to the management team; follows a few indicators.

Training spend with no visible link to the strategic objectives; results of the previous plan hard to prove; ISO 7101 audits that expose competence gaps.

A prioritized plan that serves the strategy, a measurable return, and a management team able to lead the change.

## Manager — director, head of department, process owner

Balances daily operations and improvement projects; prefers practical, short formats; relies on key people in the team.

Team absent from the job during long trainings; skills gaps on critical activities of Clinical Care & Nursing; processes not documented or without an owner.

Trainings that solve the function's real problems, applied on the job the following week, with indicators the manager can follow.

## Team member — Clinical Care & Nursing

Learns by doing and from peers; uses a phone more than a computer; wants to know what changes in daily work.

New tools introduced without explanation; theoretical trainings far from the tasks; fear of making mistakes with AI triage and clinical decision support.

Practical skills recognized by a certificate, less rework, and a clear path to the next position.

# Paths, modules and content assets

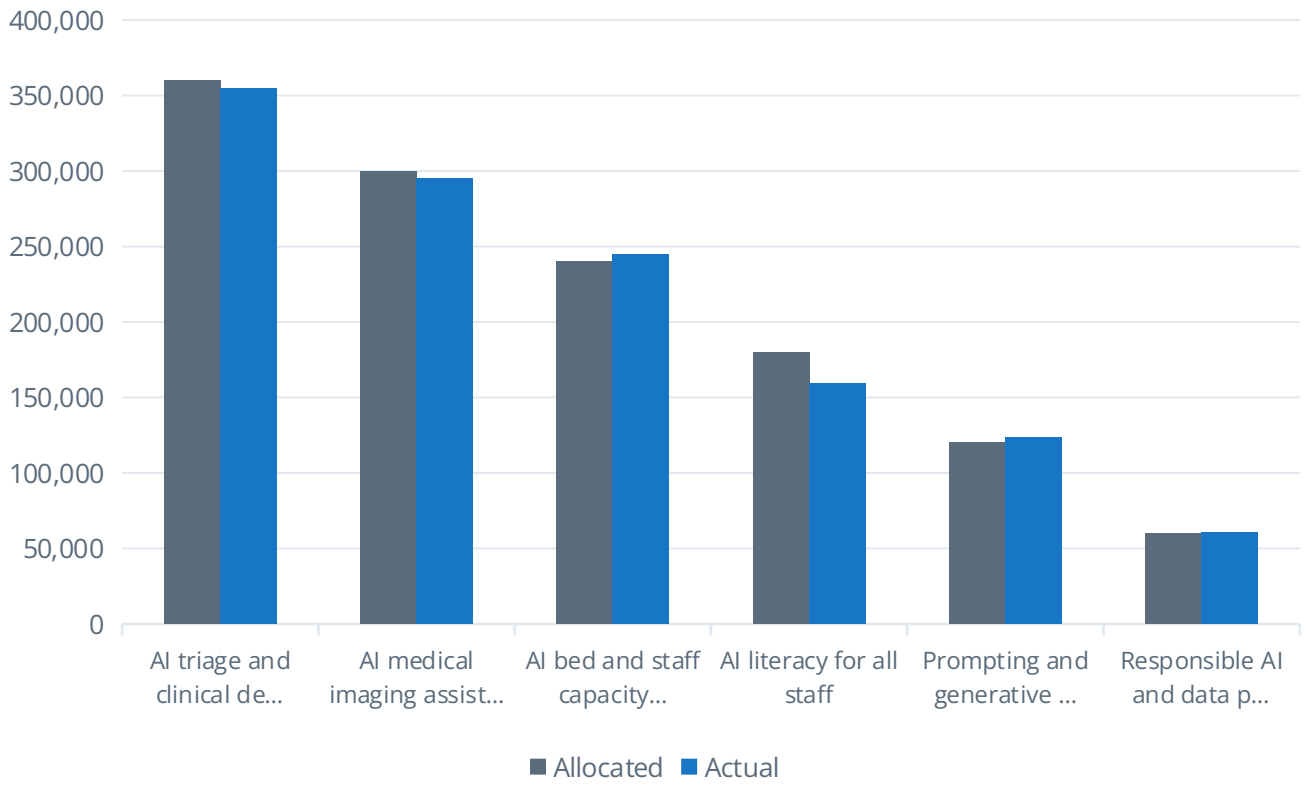
| Learning path | Days | Module | Modality | Hours | Design |
|---|---|---|---|---|---|
| AI triage and clinical decision support | 3 | M1 AI triage and clinical decision support | E-learning | 2 | Approved |
| | | M3 AI triage and clinical decision support | Virtual classroom | 3.5 | Approved |
| AI medical imaging assistance | 2 | M2 AI medical imaging assistance | Classroom | 7 | Approved |
| | | M4 AI medical imaging assistance | Classroom | 7 | Approved |

| Content | Format | Size (MB) | Review date | Status | Language |
|---|---|---|---|---|---|
| M1 material | SCORM | 42 | 2027-08-03 | Published | EN |
| M2 material | Video | 180 | 2027-04-05 | Published | FR |
| M3 material | PDF | 3.2 | 2026-12-06 | Published | AR |
| M4 material | Capsule | 12 | 2026-08-08 | Published | FR |

*Source: TER sections 5.3 and 5.4.*

# Allocated, committed, spent and refunds expected

Allocated 1,260,000, committed 1,260,000 (100 %), spent 1,238,497 (98 % of planned); expected refunds 352,800.




*Allocated (Muted) versus actual (Azure) by training, (TER 5.7).*

| Line | Actual | Refund |
|---|---|---|
| AI triage and clinical decisi... | 354,598 | 100,800 |
| AI medical imaging assistance | 295,234 | 84,000 |
| AI bed and staff capacity pla... | 244,886 | 67,200 |
| AI literacy for all staff | 159,533 | 50,400 |
| Prompting and generative AI a... | 123,413 | 33,600 |
| Responsible AI and data prote... | 60,833 | 16,800 |
| **Total** | **1,238,497** | 352,800 |

| Provider | Accredited until | /5 |
|---|---|---|
| Atlas Formation Conseil | 2027-04-25 | 4.2 |
| Maghreb Learning Partners | 2027-08-03 | 3.8 |

05

# Delivery, evaluation and audit

Sessions, Kirkpatrick levels, ROI, internal audit and lessons learned

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# Four sessions delivered with their enrolments

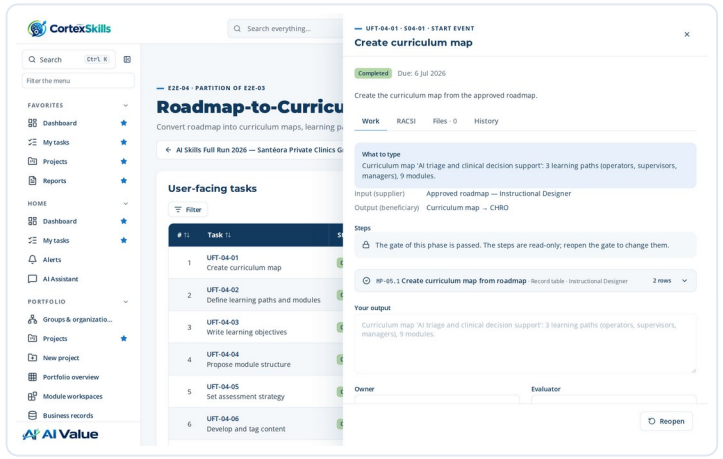
| Session | Date | Capacity | Trainer | Enrolled |
|---|---|---|---|---|
| S-01 AI triage and clinical decision support | 2026-07-19 | 12 | Zineb Kettani | 3 |
| S-02 AI medical imaging assistance | 2026-07-26 | 12 | Zineb Kettani | 3 |
| S-03 AI triage and clinical decision support | 2026-08-02 | 12 | Zineb Kettani | 3 |
| S-04 AI medical imaging assistance | 2026-08-09 | 12 | Zineb Kettani | 3 |

**10 / 12**

learners present at their session

**10 / 12**

enrolments completed




*A task of the delivery process with its typed steps.*

EVALUATION

# Results measured at three Kirkpatrick levels

**77 %**
1 — Reaction

**74 %**
2 — Learning

**75 %**
3 — Behaviour

| Level | What is measured | When | Instrument | Results |
|---|---|---|---|---|
| 1 — Reaction | Satisfaction and perceived usefulness | End of each session | Satisfaction questionnaire | 2 × 77 % |
| 2 — Learning | Knowledge and skills acquired | End of the training | Pre- and post-test, case study | 2 × 74 % |
| 3 — Behaviour | Use of the skills at the workstation | Three months after | Manager observation grid | 2 × 75 % |
| 4 — Results | Effect on the indicators of the function | Six to twelve months after | Indicators of the function (KPI) | — |

Certifications (TER 5.10): 4 valid, 1 expiring, 1 expired — the expired one triggers an alert and a renewal session.

# Phillips ROI and the indicators of the ROI study

ROI and business impact report — Santéora Private Clinics Group

**108**
Training ROI

**4.5**
Average Level 1 satisfaction

**75.6**
Skill progression rate

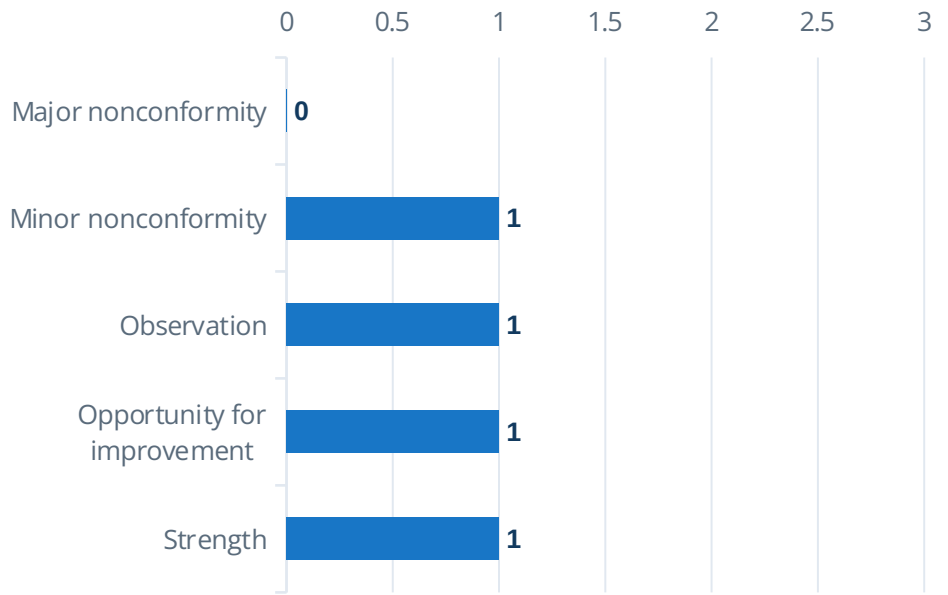
**4.1**
Budget execution variance

| ID | Indicator | Formula | Target | Latest value | Status |
|---|---|---|---|---|---|
| KPI-037 | Training ROI | (Net programme benefits / programme costs) x 100 (Phillips) | ≥ 100% | 108 (2026-10) | Green |
| KPI-012 | Average Level 1 satisfaction | Mean reaction score across closed sessions (1-5) | ≥ 4.2 | 4.5 (2026-10) | Green |
| KPI-013 | Skill progression rate | Learners gaining at least one level within 6 months / learners traine… | ≥ 70% | 75.6 (2026-10) | Green |
| KPI-022 | Budget execution variance | \|Actual - allocated\| / allocated x 100 | ≤ 5% | 4.1 (2026-10) | Green |
| KPI-023 | OFPPT refund recovery rate | Refunds received / refunds forecast x 100 | ≥ 90% | 97.2 (2026-10) | Green |

*Source: ROI and business impact report (DT-E2E-08), indicators section.*

# Audit report — Santéora Private Clinics Group

Conforming with one minor nonconformity to correct within 60 days.




*Findings by grade.*

| ID | Grade | Requirement | Statement |
|---|---|---|---|
| F-1 | Minor nonconformity | ISO 7101 7.2 | Two trainers have no evidence of competence on file. |
| F-2 | Observation | ISO 7101 9.1 | Level 3 evaluations are planned but not yet scheduled. |
| F-3 | Opportunity for improvement | ISO 7101 7.1.6 | Capture lessons learned at the end of each session. |
| F-4 | Strength | ISO 7101 7.2 | Needs analysis fully traced from strategic objectives. |

Strengths: Clear needs analysis; strong management involvement; complete attendance records.
Follow-up: Verification of the corrective action at the next steering committee.

# REX — AI triage and clinical decision support

### What went well

The stakeholder register and multi-channel distribution lifted the response rate.

### What did not

SWOT inputs arrived late from one function.

### Root cause

No backup nominated for the function head.

### What we will do next time

Name a backup for every function head at kickoff.

06

# Documents produced

37 documents published in English, each traced to its data

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# Every end-to-end process leaves its document

All published, versioned and approved by a second person — 1,077 pages.

| Code | Document | Pages |
|---|---|---|
| DT-E2E-01 | Annual L&D plan and budget | 29 |
| DT-E2E-02 | Skills gap diagnosis | 23 |
| DT-E2E-03 | Training needs report and roadmap | 29 |
| DT-E2E-04 | Curriculum design document | 25 |
| DT-E2E-05 | Training delivery report | 29 |
| DT-E2E-06 | Learner engagement report | 30 |
| DT-E2E-07 | Training evaluation report | 24 |
| DT-E2E-08 | ROI and business impact report | 24 |
| DT-E2E-09 | Compliance and certification register | 22 |
| DT-E2E-10 | Internal mobility and career paths | 26 |

| Code | Document | Pages |
|---|---|---|
| DT-E2E-11 | Onboarding programme record | 22 |
| DT-E2E-12 | Knowledge capture and transfer record | 23 |
| DT-E2E-13 | Content catalogue and lifecycle | 23 |
| DT-E2E-14 | Extended enterprise training report | 23 |
| DT-E2E-15 | Data, AI and integration governance | 24 |
| DT-E2E-16 | Vendor management file | 22 |
| DT-E2E-17 | Change and adoption plan | 23 |
| DT-E2E-18 | Quality, risk and improvement report | 30 |
| DT-E2E-19 | Consulting engagement closure report | 26 |
| DT-E2E-20 | Multi-axis diagnostic and maturity report | 27 |

# Documents produced (continued)

| Code | Document | Pages |
|---|---|---|
| DT-E2E-21 | Previous plan impact evaluation | 25 |
| DT-E2E-22 | Multi-site deployment plan | 24 |
| DT-E2E-23 | Regulatory submission file | 22 |
| DT-E2E-24 | Competitive intelligence note | 23 |
| DT-E2E-25 | Job architecture and competency map | 23 |
| DT-E2E-26 | Forward planning of jobs and skills (GPEC) | 25 |
| DT-E2E-27 | Training demand consolidation | 22 |
| DT-E2E-28 | Performance assessment and RAG dashboard | 22 |
| DT-E2E-29 | Functional SWOT and improvement axes | 23 |
| DT-E2E-30 | Prioritized resource plan | 24 |

| Code | Document | Pages |
|---|---|---|
| DT-E2E-31 | Post-engagement roadmap | 24 |
| DT-E2E-32 | Scope of work and stakeholder register | 31 |
| DT-E2E-33 | Questionnaire campaign report | 30 |
| DT-TER | Training Engineering Report and Training Plan | 193 |
| DT-PLAN | Training plan with detailed agendas | 35 |
| DT-AUDIT | Audit report | 5 |
| DT-MASTER | Master list of documented information | 22 |

TRACEABILITY

# Every number of the report comes from a record

Every table of this document is drawn from the application's records; data as of 07/10/2026, 15:27:56.

| Source | Records | Process | Last change |
|---|---|---|---|
| Questionnaire responses | 30 | MP-54 | — |
| Stakeholder | 40 | MP-53 | 2026-10-07 |
| ScopeCriterion | 17 | MP-52 | 2026-10-07 |
| SkillAssessment | 12 | MP-16 | 2026-10-07 |
| SkillGap | 8 | MP-16 | 2026-10-07 |
| TrainingNeed | 10 | MP-03 | 2026-10-07 |
| RoadmapInitiative | 4 | MP-04 | 2026-10-07 |
| WbsNode | 3 | MP-49 | 2026-10-07 |
| Session | 4 | MP-05 | 2026-10-07 |
| TrainingPlan | 1 | MP-49 | 2026-10-07 |
| Employee | 16 | MP-16 | 2026-10-07 |
| StrategicObjective | 2 | MP-01 | 2026-10-07 |

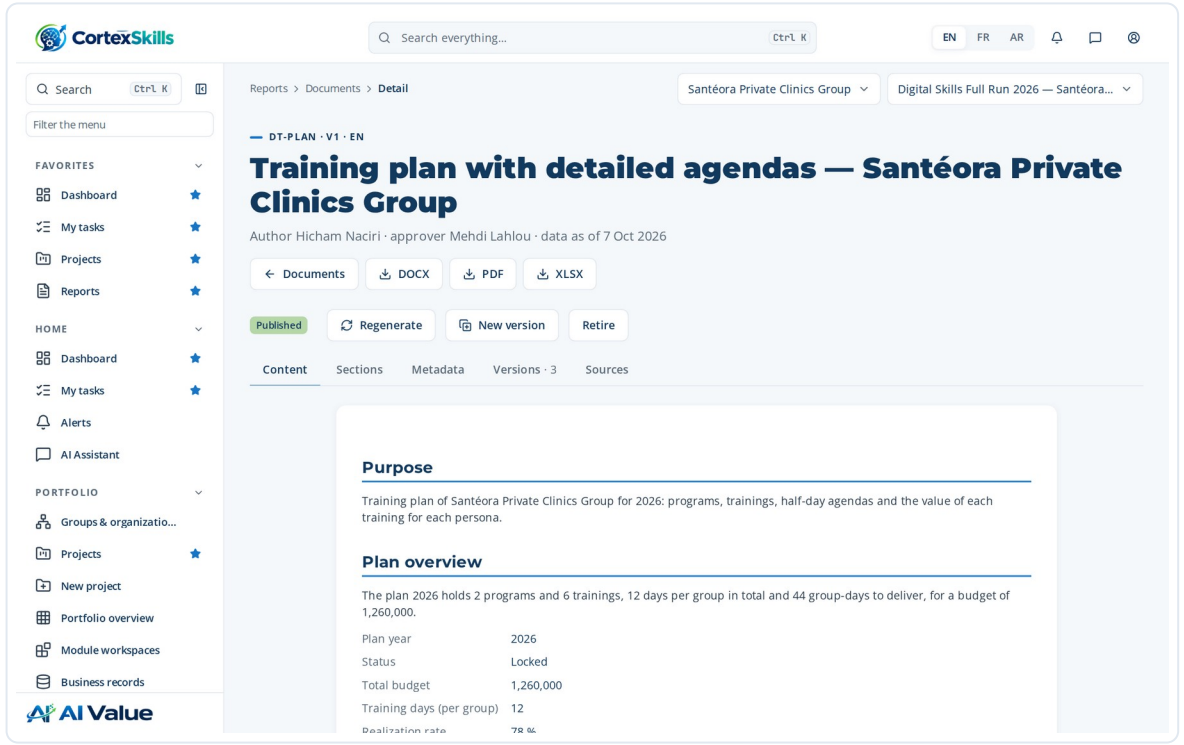
## Governance of the documents

Author Hicham Naciri, approver Mehdi Lahlou: two different people.

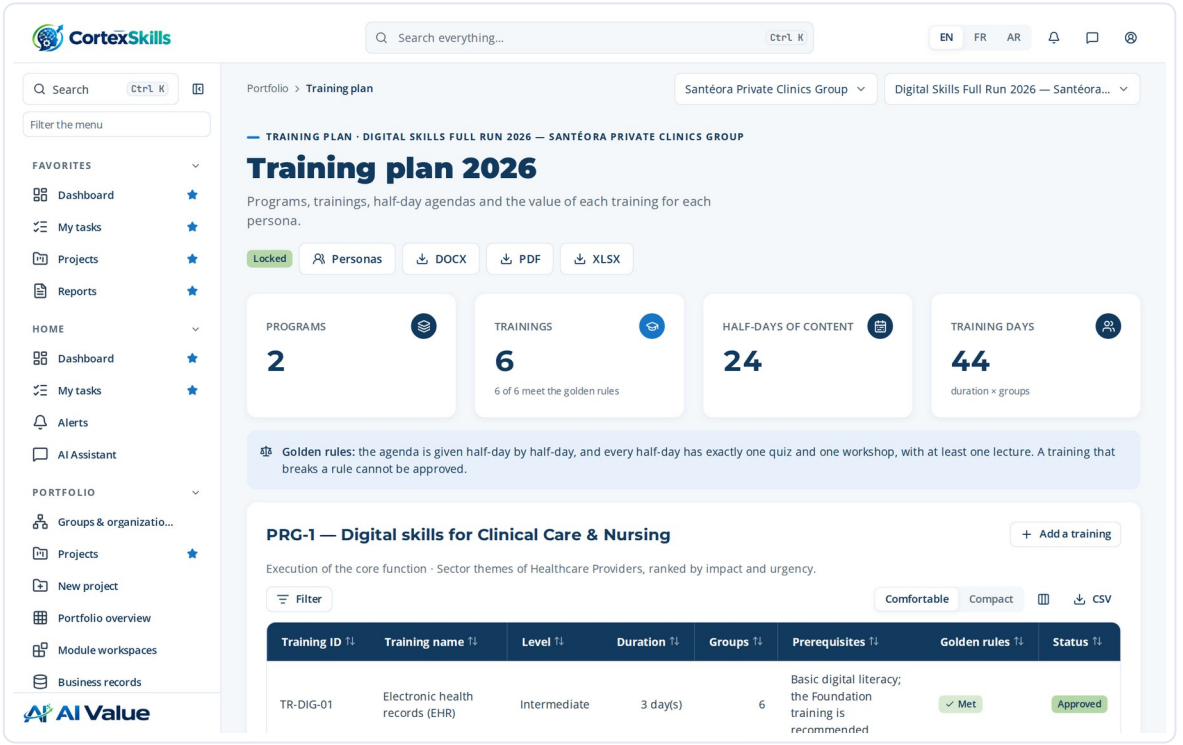
Each version is kept; a change of the data marks the document as stale.

20 data sources listed with their record counts and process.

# The Training Plan as the users see it

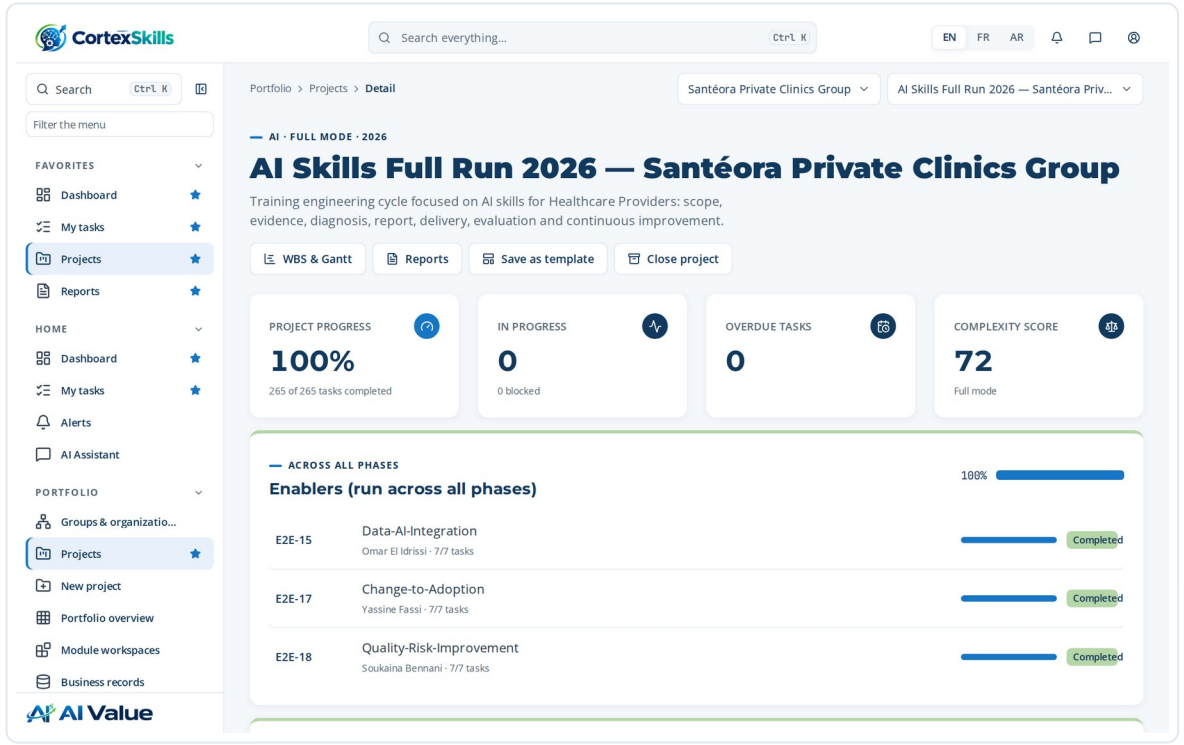



*Training plan with detailed agendas — Santéora Private Clinics Group*

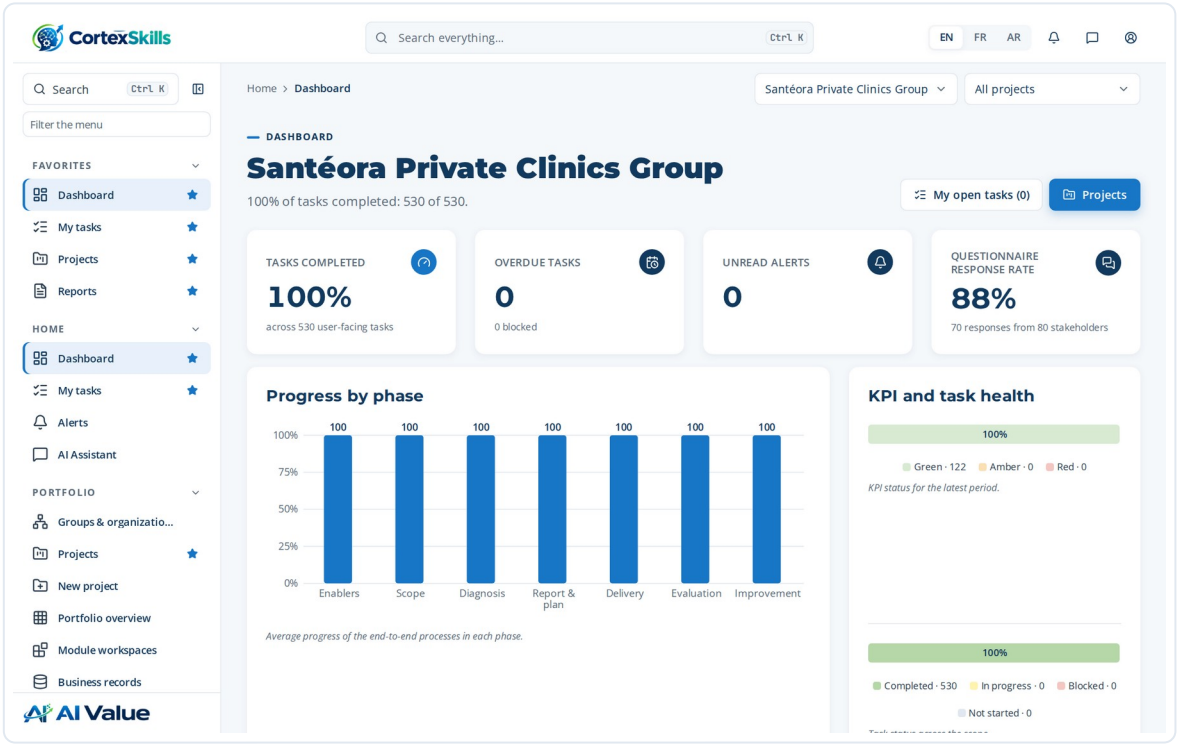



*Training plan screen: programs, trainings and sessions.*

# The project workspace and the dashboard




*Project workspace: phases, processes and progress.*




*Dashboard of the clinic group.*

CONCLUSION

# What the clinic group has at the end of the cycle

- A Training Engineering Report of 193 pages, built from 35 answers, 40 stakeholders and the diagnosis of every function.
- A locked Training Plan: 6 trainings, half-day agendas, a value proposition per persona and a budget of 1,260,000
- Evaluation at three Kirkpatrick levels, an ROI study, followed certifications and an internal audit with one minor nonconformity to correct.
- 37 published documents ready for an accreditation audit (ISO 7101, JCI), each traced to the data and the step that produced it.
- Next: waves 2 and 3 of the roadmap and the level-4 (results) evaluation six to twelve months after the trainings.

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MEASURABLE • GOVERNED • SUSTAINABLE

# Thank you

MEASURABLE · GOVERNED · SUSTAINABLE